June 16, 2025

VIA Electronic Submission to https://www.regulations.gov

Stephanie Carlton, Deputy Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS–0042–NC,
P.O. Box 8013, Baltimore, MD 21244–8013

Re: Request for Information; Health Technology Ecosystem. (RIN 0938–AV68, CMS–0042–NC)

The American Health Care Association and National Center for Assisted Living (AHCA/NCAL) represents over 15,000 long term and post-acute care facilities, or 1.1 million skilled nursing facility (SNF) beds and over 300,000 assisted living (AL) beds. With such a membership base, the Association represents the majority of SNFs and a rapidly growing number of assisted living communities as well as residences for individuals with intellectual and developmental disabilities (ID/DD). We appreciate the opportunity to comment on the *Request for Information; Health Technology Ecosystem*.

We agree that effective and responsible adoption of technology has the potential to empower patients and their families and caregivers to make informed decisions for their health and well-being and welcome this discussion of the market of digital health products for Medicare beneficiaries as well as the state of data interoperability and broader health technology infrastructure. We believe that it is imperative that the federal government provide leadership, governance, and support to encourage more widespread adoption of accessible, safe, secure, and user-friendly interoperable digital health technology for providers and their patients. The Centers for Medicare & Medicaid Services (CMS) and the Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC) are best positioned to provide the leadership to facilitate infrastructure progress enable beneficiary access to effective digital capabilities needed to make informed health decisions, and to increase data availability for all stakeholders contributing to health outcomes.

We are eager to work with CMS and ASTP/ONC in identifying a pathway to finding solutions that are attainable to all stakeholders across the information health technology ecosystem. If you have questions about any of our comments, please contact Daniel Ciolek at dciolek@ahca.org.

Sincerely,

Daniel E Ciolek
Associate Vice President, Therapy Advocacy

Detailed AHCA/NCAL Comments

1. SNF/NF, AL, and ID/DD Provider Setting Profiles and the Populations of Individuals Served

Of importance, and relevant to our comments, is that unlike other healthcare facility-based provider settings that address acute and sub-acute conditions for a limited time-window, most of the individuals our members care for are long-term residents – it is their home. Except for a portion of SNF short-stay post-acute patients who require certain nursing and/or rehabilitation therapy services to return to the community, most people in nursing facilities (SNF/NF) and those in AL or ID/DD residences require residential care to manage chronic conditions reflected through physical and/or cognitive impairments.

Regarding the long-term and post-acute care (LTPAC) provider settings represented by our membership, we first describe the types of healthcare and healthcare-related services they furnish and the characteristics of the residents they serve. Next, we highlight the diversity of payor sources since a provider's reliance on fixed federal and state payors impacts a provider's capacity to assume the significant hardware, software, and administrative costs associated with interoperable digital health technology. Finally, we share information regarding the diversity of the resident population across the SNF, AL, and ID/DD provider community as a reflection of the digital health needs footprint of these residents within the health information technology (HIT) ecosystem.

Most of the 14,700 SNFs nationwide serve a dual purpose with 93 percent being dually certified for furnishing Medicare and Medicaid services. As mentioned above, SNFs provide short-term post-acute services to patients who require skilled nursing and/or rehabilitation services on an inpatient basis. Additionally, SNF's/NFs furnish long-term care for residents requiring 24/7 care due to various medical, mobility, activities of daily living, cognition, and behavioral challenges that cannot be addressed adequately in the community. Health care services from various professions may be furnished to residents in the facility, at the offsite office or facility, or via telehealth and remote patient monitoring (RPM) technology. While a small number of SNFs serve pediatric populations, the vast majority serve adults with chronic conditions and related disabilities, predominantly for adults over the age of seventy-five. Overall, Medicaid accounts for sixty-three percent of SNF revenues while private and other payers represent twenty-four percent, and Medicare only thirteen percent.[1]

According to the MedPAC March 2024 Report to Congress[2] SNF facilities range from small to large organizations and operate on minimal net margins:

- **The SNF all-payer profit margin was a negative 1.4 percent.**
- **The median SNF had 100 beds, while ten percent of facilities had 176 or more beds and ten percent of facilities had 50 or fewer beds.**

There are approximately 30,600 AL communities that serve older individuals or those with limited disabilities who typically need help with everyday activities and some health care services but do not require 24-hour skilled nursing services for extended periods of time[3]. These communities offer a unique mix of companionship, independence, privacy, and security in a home-like setting. The philosophy of assisted living is built on the concept of delivering person-centered care and services to each individual resident. Some assisted living communities specialize in serving individuals with specific needs. These may include, but are not limited to,

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[1] *A Look at Nursing Facility Characteristics Between 2015 and 2024*. KFF. https://www.kff.org/medicaid/issue-brief/a-look-at-nursing-facility-characteristics/ [accessed March 6, 2025]

[2] March 2024 Report to Congress: *Medicare Payment Policy: Chapter 6: Skilled Nursing Services*. Medicare Payment Advisory Commission

[3] Assisted Living Facts & Figures. AHCA/NCAL. https://www.ahcancal.org/Assisted-Living/Facts-and-Figures/Pages/default.aspx [accessed March 6, 2025]

Alzheimer's disease or other forms of dementia (such as memory care units), intellectual and developmental disabilities, and particular medical conditions (e.g., Parkinson's disease) or other needs. AL communities do not directly provide certain health care services, such as physical therapy or pharmacy, but work with other providers to offer these services.

- **AL services are not included in Medicare benefits, and Medicaid does not cover AL room and board, so most costs are private-pay with only one in five residents being eligible for Medicaid home and community-based services (HCBS) such as personal care and supportive services.**
- **Forty-three percent of AL residents are small and serve 4-10 residents at a time, while only ten percent have 100 or more residents.**

There are also over 5,300 ID/DD residences, or Intermediate Care Facilities for Individuals with Intellectual Disabilities (ICF/IID) serving over 56,000 residents of all ages daily, with over seventy-five percent being between the ages of 22 and 65[4]. Many of these individuals are non-ambulatory, have seizure disorders, behavior problems, mental illness, visual or hearing impairments, or a combination of the above. ICFs provide active treatment and services for people with significant support needs. They offer 24-hour supervision, health care, therapies, activities, and training intended to maximize residents' autonomy and independence.

- **Virtually all funding for ID/DD residents is under Medicaid benefits.**
- **The average number of beds per residence is 14, ranging from 9 for private ID/DD residences to 52 for public residences.**

2. Digital Capabilities Across SNF/NF, AL, and ID/DD Provider Settings

All of the care provider settings represented by our membership have access to and store PHI. Of these, many store this information electronically to varying degrees and generally have limited interoperable connectivity in the current digital health technology ecosystem. The variations in digital capabilities across our post-acute and long-term care provider membership is directly the result of historical federal prioritization of hospital inpatient and primary care interoperability support through the implementation of the Health Information Technology for Economic & Clinical Health (HITECH) Act of 2009 and in subsequent coordinated care, bundled care, and other integrated care value-based payment models advanced by CMS. Since post-acute and long-term care providers were not specifically identified in the HITECH Act, the Federal administrative support necessary to facilitate seamless secure interoperable electronic exchange of the patient information between providers, and to the patient, particularly at transitions of care, has been quite limited.

In the absence of a Federal coordinated effort to support IT integration, LTPAC providers that have adopted HIT have instead developed non-standardized HIT, including electronic health records (EHR) to support setting-specific clinical and operational activities. In addition to EHR systems, many of the SNF, AL and ID/DD providers in our membership utilize other digital technologies to support resident care. Telehealth technologies are used to expand beneficiary access to care and reduce unnecessary hospitalizations. Remote patient monitoring (RPM) and sensor technologies can provide valuable information to improve resident safety and more timely and effective care interventions. Clinical decision support systems (CDSS) including artificial intelligence (AI) driven analytics help to improve treatment decisions and outcomes. Resident care management activities are also supported by tools for purposes such as electronic nursing documentation, medication administration, incident reporting and quality assurance, and resident engagement tools such as patient portals and apps. HIT tools are also used to facilitate communication with various internal and external clinical support

[4] AHCA/NCAL internal analysis of the following public and proprietary data sources: A-Certification and Survey Provider Enhanced Reporting (CASPER) data. B-Medicaid Budget and Expenditure System/State Children's Health Insurance Program Budget and Expenditure System (MBES/CBES). C-IMPLAN Group LLC, IMPLAN System (data and software)

services including pharmacy, laboratory, radiology, rehabilitation therapy professionals, and for administrative purposes such as scheduling, claim processing, compliance programs and more.

However, the current digital ecosystem for many of these technologies today is fragmented and interconnectivity when present is commonly built upon customized digital 'work arounds' which is a significant barrier to attaining the level of effective digital capabilities needed to support patients, families and their caregivers in making informed health decisions, and for provider stakeholders to improve health outcomes.

For example, as recently as December 23, 2023, the Office of the Assistant Secretary for Planning and Evaluation (ASPE) within HHS published a report titled *Health Information Technology Adoption and Utilization in Long-Term and Post-Acute Care Settings*[5] which found that most LTPAC providers, including SNFs, have generally adopted electronic health records EHR to support clinical and business needs at a rate comparable to hospitals and primary care providers, but adoption of EHR in residential care settings such as AL and ID/DD is limited.

> ***"Yet despite the lack of a federally-funded program and policy requirements, estimates of EHR adoption rates among nursing home and SNF providers, as well as HHAs, were greater than 78% in 2018, which is on par with EHR adoption in office-based primary care settings. Residential care settings were estimated to be much lower overall, at 26% -- higher than that for larger facilities and much lower for small facilities."***

However, the authors also emphasized that interoperable exchange of health information is not routine or widely used. In other words, LTPAC providers utilize their internal EHR, but modernization to improve interoperability capabilities remains slow without focused and realistic policy levers. Despite barriers, the authors concluded that there are opportunities for emerging policies to support secure interoperability in LTPAC.

The breadth of this digital divide was reflected in a recent May 2024 ONC Data Brief titled *Interoperable Exchange of Patient Health Information Among U.S. Hospitals: 2023*[6].

> **Specifically, ONC indicated that only <u>17 percent</u> of hospitals are able to routinely send interoperable health information to LTPAC providers and only <u>8 percent</u> of hospitals were able to routinely receive such information from LTPAC providers.**

While we appreciate the recent ONC efforts in late 2023 in launching the Trusted Exchange Framework and Common Agreement (TEFCA) to enable nationwide health information exchange that could provide opportunities for LTPAC providers with nonstandard health IT to exchange information through a Health Information Exchange (HIE), a recent August 2024 Health Affairs Scholar article titled *The state of health information organizations and plans to participate in the federal exchange framework*[7] has revealed that this alternative method for information exchange will likely be insufficient to resolve the interoperability gap for LTPAC providers as 32 percent of the current Health Information Organizations (HIOs) that could facilitate this information exchange through their HIEs have indicated they may not participate in TEFCA. As the authors summarized,

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[5] Health Information Technology Adoption and Utilization in Long-Term and Post-Acute Care Settings. Office of the Assistant Secretary for Planning and Evaluation (ASPE) at the U.S. Department of Health & Human Services by RTI International. December 2023.

[6] Interoperable Exchange of Patient Health Information Among U.S. Hospitals: 2023. Office of the National Coordinator for Health Information Technology (ONC). May 2024.

[7] Jordan Everson, Wei Chang, Vaishali Patel, Julia Adler-Milstein, The state of health information organizations and plans to participate in the federal exchange framework, *Health Affairs Scholar*, Volume 2, Issue 8, August 2024

"While TEFCA appears to have successfully engaged the majority of HIOs, achieving nationwide exchange will require policy efforts to either attract the remaining HIOs or ensure that nonparticipating HIOs' providers have another option for TEFCA participation."

Since the enactment of the HITECH Act AHCA/NCAL and other LTPAC Associations and the LTPAC Health IT Collaborative[8] have commented on various Agency HIT programs and have sought the support of HHS both in funding and policy. HHS has provided limited support to support secure HIT interoperability and cybersecurity within the constraints of the current statutory framework, but we believe more is necessary.

Specific RFI Sections and Questions

Providers - Digital Health Apps

PR–1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC–5) digital health products for their patients?

a. What are the current obstacles?

AHCA /NCAL Comment

There are numerous environmental and patient-specific obstacles that CMS and its partners need to be considered and to mitigate including the following:

- General broadband access for the provider or the patient in certain geographic locations needs to be sufficient to transmit and receive health information.
- Architectural barriers within buildings prevent or disrupt Wi-Fi or Bluetooth transmissions between such digital health devices and a centralized EHR or other health information exchange platform needs to be resolved.
- Americans with Disabilities Act (ADA) compliant technology requirements are necessary to meet the needs of those older adults and persons with visual, auditory, cognitive, and physical disabilities, including persons living in ANF, AL, or ID/DD residences.
- Patient/Caregiver physical and cognitive impairments that may impact the person's ability to use or the effectiveness of digital health devices.
- Patient/Caregiver privacy concerns about digital health sensor technology transmitting information via automation, particularly video technology.
- Patient/Caregiver concerns about the safety of their digital health information being transmitted safely and securely to the intended healthcare entity.
- Patient/Caregiver limitations in health literacy.
- Patient/Caregiver limitations in technology literacy.
- Patient/Caregiver limitation in the ability to afford computer or remote app technologies necessary to support specific digital health devices in transmitting information to the intended receiver of the health information.
- The complexity of different processes for multi-factor authentication to log onto multiple digital health products.
- The absence of substantial interoperability capabilities of most LTPAC HIT platforms and digital health technologies involved is a barrier to a patient's (or family/caregiver's) access to a digital care plan as they navigate the health system. For example, technology that would allow a provider or patient to update or verify the accuracy of the patient's medication profile, physician orders, or patient's care preferences available to providers in transition of care and emergency situations is essential to patient safety.

[8] LTPAC Health IT Collaborative: https://www.ltpachit.org/

b. What information should providers share with patients when using digital products in the provision of their care?

AHCA /NCAL Comment

Optimally, once various barriers to accessing digital health technology interoperability are resolved, patients should be able to access the types of health information about themselves digitally that they currently are able to access through traditional manual process including photocopies, photos, physical x-rays, and files downloaded to digital storage devices (e.g., CDs, flash drives). Such access would also benefit providers by reducing the administrative burden and costs associated with retrieving and sharing such information manually. In addition, patients, families, and caregivers should be informed about various digital health products to a level of detail sufficient enough for the individual to be able to make an informed choice to consent to or not consent to its use.

The following are several topics that may be considered for inclusion in information a provider might need to share with a patient before using certain digital products:

- What is the clinical purpose for the digital product?
- What patient internet capabilities/technologies are necessary to support the use of specific digital health technology?
- Whether the patient has the right to refuse specific digital health technology.
- Whether the technology is Health Insurance Portability and Accountability Act (HIPAA) compliant, and the potential implications if it is not.
- Whether the technology is ADA compliant to mitigate for a patient's (or family/caregiver's) cognitive or physical impairments.
- How does a patient obtain or access specific technology?
- What patient costs are associated with the specific technology?
- How is the specific technology used/operated by the patient, family, or caregiver?
- Where does the patient, family, caregiver go for troubleshooting/technical support?

c. What responsibilities do providers have when recommending use of a digital product by a patient?

AHCA /NCAL Comment

In addition to the types of information we discuss that might need to be shared in our response to question 'b' above, we believe it may be important for a provider to disclose any financial relationship or other potential conflicts of interest with the recommended digital health technology.

PR–3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? Please address all of the following:

a. Current challenges in accessing different data formats.
b. Impact on patient care quality.
c. Technical barriers to full data accessibility.
d. Cost or privacy implications of making all data formats interoperable.
e. Priority level compared to other interoperability needs.

AHCA /NCAL Comment

Our members do not believe that a 'one-size-fits-all' solution that could incorporate all these capabilities is technically or financially feasible and could possibly add to the burden, reduce software performance, and

reduce care quality due to overload. Such technical capabilities should be modular in nature and permit financially feasible scalable increases in capabilities needed to address the data recipient's needs as a patient, provider, insurer, public agency, researcher, or for other legally permissible uses. Also, we are concerned about the potential impact on patient privacy associated with full data accessibility, particularly for data unrelated to the data recipients need to know. In some cases, there are laws related to health information that prohibit full access to certain information.

Providers - Data Exchange

PR–5. Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

a. ***Patient Access API.***
b. ***Standardized API for Patient and Population Services.***
c. ***Provider Directory API.***
d. ***Provider Access API.***
e. ***Payer-to-Payer API.***
f. ***Prior Authorization API.***
g. ***Bulk FHIR—Do you support Group ID-based access filtering for population[1]specific queries?***
h. ***SMART on FHIR—Do you support both EHR-launched and standalone app access? What does the process for application deployment entail?***
i. ***CDS Hooks (for clinical decision support integrations).***

AHCA /NCAL Comment

Our AHCA/NCAL member SNF, AL, and ID/DD providers would benefit from some or all of the listed FHIR APIs and capabilities to improve clinical documentation and workflows, improve communication with care partners and patients/families/caregivers, and improve administrative quality reporting, prior authorization, claims processing, and other financial transaction activities. However, we emphasize again that at present, many SNF, AL, and ID/DD providers do not have the size and necessary human and financial resources, or technical expertise to be able to effectively implement such interoperable HIT capabilities, including FHIR APIs without support. Many providers remain without an EHR system let alone the capability to receive or transmit health records electronically or offer more advanced capabilities such as patient portals or apps.

Currently, providers are often required to manually log onto their care partner or payer applications to complete transactions as opposed to more efficient and less burdensome automated processes enabled through FHIR APIs. When a provider offers a patient portal or app, it is often via a proprietary or customized non-standardized application. Again, this is the result in part due to factors such as limited or no broadband access and lack of financial support or incentives and a lack of coordinated federal governance to support more widespread adoption of interoperable technology. AHCA suggests that CMS should recognize that most users of digital health information or that make operational decisions to obtain and deploy digital health capabilities in their healthcare organization are not technology experts, nor do smaller providers have the ability to employ chief information officers or IT experts to help navigate the digital health ecosystem. CMS guidance needs to consider approaches to adequately educate provider technology users and technology purchasing decision makers about such issues in plain language and not just in technical language only familiar to the IT industry.

As part of the process to support more widespread adoption of digital technology solutions such as FHIR APIs in SNF, AL, and ID/DD settings we believe that CMS should be intentional and focus on incremental approaches in adopting those policy and health information technology specifications that are necessary to support the transition of sharing health information for clinical, quality measure, and public health reporting and other needs from current paper-based or other manual processes or customized digital 'work arounds' to

less burdensome and more automated electronic processes. Our AHCA/NCAL member providers have various levels of digital capabilities, potential digital data exchange partners, EHR and other HIE vendor platforms, resident population care needs, technology expertise, and financial and human resource availability. A 'one-size-fits-all' interoperability advancement strategy would be doomed to failure.

We also believe that CMS should consider the complexity of digital relationships SNF providers have with healthcare delivery partners (e.g., therapy, pharmacy, laboratory, payers, public agencies). Unlike hospitals systems that have internal chief information officers (CIOs), dedicated IT personnel, and the resources and capacity to incorporate such ancillary services within their core operations and EHR systems, most SNFs lack such resources or capacity and rely on securing such services from external providers through arrangements. These external care partners were also excluded from the federal interoperability support through the HITECH Act and following incentive programs and also often developed uncertified EHR systems that cannot communicate with the SNF EHR without expensive customized digital 'work-arounds.' As CMS contemplates efforts to improve SNF interoperability readiness and capabilities, the Agency must incorporate strategies to ensure that those external care partner interoperability capabilities are also assessed and ultimately supported.

Another critical factor is that CMS should recognize differences in patient populations and care needs across the nursing home and entire post-acute and long-term care sector and be flexible and not prescriptive in establishing which specific data elements or data exchange compatibility purposes must be demonstrated for a future performance incentive or disincentive. One of the reasons most SNFs do not use certified EHR module technology today is that the minimum interoperability certification requirements were established by and for hospitals and primary care providers subject to HITECH Act regulations. The current certification requirements reflect care workflows that are not relevant to SNF needs, and many critical SNF workflows are not addressed, which again raises provider cost to purchase software 'work-arounds' to securely share information interoperably.

FHIR APIs may provide a more reasonable pathway to allow for the necessary flexibility to address these challenges to advancing provider digital capabilities as opposed to the more rigid historical certification process. Co-founded by CMS and the MITRE Corporation, and hosted by HL7, PACIO brings together providers, standards organizations, technology vendors, payers, patients, and advocacy groups. Its mission is to establish FHIR-based technical implementation guides and reference implementations to support use case-driven APIs that enable standardized, patient-centered data exchange.

PACIO's work is especially impactful because it directly addresses the SNF and other post-acute and long-term care sector's unique needs and limitations. The project focuses on integrating CMS assessment data elements from the Data Element Library into health IT systems through FHIR APIs, facilitating the electronic exchange of key information like cognitive status, functional status, care plans, and patient goals. This work supports better care transitions, improved coordination, and more efficient sharing of vital health information across disparate systems.

However, the success of such initiatives depends on resolving broader, sector-wide barriers. These include the lack of a uniform technological baseline, uneven workforce capacity, and a shortfall of financial and policy incentives tailored to LTPAC. While PACIO represents a critical step forward, foundational elements such as shared standards, stable infrastructure, and a mutual understanding of how to operationalize data exchange must be established and sustained for interoperability to take root systematically.

PR–6. Is TEFCA currently helping to advance provider access to health information?

a. Please provide specific examples.
b. What changes would you suggest?
c. What other options are available outside of TEFCA?

d. Are there redundant standards, protocols or channels or both that could be consolidated?

AHCA /NCAL Comment

Though the Trusted Exchange Framework and Common Agreement (TEFCA) has only been operational since December 2023, nine designated Qualified Health Information Networks (QHINs) were approved as of April 2025. According to the Sequoia Project and the Recognized Coordinating Entity (RCE), 1,633 organizations were live on TEFCA (QHINs, participants, and subparticipants) and to the framework agreement as of January 2025. RCE, through its collaboration with ASTP/ONC, is responsible for developing, implementing, and maintaining the Common Agreement.

We agree that TEFCA is currently helping advance provider access to digital health information and reducing the digital divide between those hospital and primary care providers that were afforded substantial support through HITECH Act and subsequent incentive programs and all other types of health care providers and their care support partners. Unlike rigid certification requirements, TEFCA offers a more flexible and nimble approach to advance digital health information exchange across the healthcare ecosystem and ultimately may be the most effective pathway for success.

However, while we appreciate the recent ONC efforts in late 2023 in launching TEFCA to enable nationwide health information exchange that could provide opportunities for LTPAC providers with nonstandard health IT to exchange information through a HIE, a recent August 2024 Health Affairs Scholar article titled *The state of health information organizations and plans to participate in the federal exchange framework*[9] has revealed that this alternative method for information exchange will likely be insufficient to resolve the interoperability gap for LTPAC providers as 32 percent of the current Health Information Organizations (HIOs) that could facilitate this information exchange through their HIEs have indicated they may not participate in TEFCA. CMS and ASTP/ONC must seek to assure that all HIEs participate in TEFCA or must otherwise provide for an alternative pathway for interoperability capability enhancement success for providers participating in a HIE that is not a TEFCA participant.

PR–7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem?

- ***How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?***

AHCA /NCAL Comment

Please see our detailed response to PR-5 above, which includes the following statement: As part of the process to support more widespread adoption of digital technology solutions such as FHIR APIs in SNF, AL, and ID/DD settings we believe that CMS should be intentional and focus on incremental approaches in adopting those policy and health information technology specifications that are necessary to support the transition of sharing health information for clinical, quality measure, and public health reporting and other needs from current paper-based or other manual processes or customized digital 'work arounds' to less burdensome and more automated electronic processes. Our AHCA/NCAL member providers have various levels of digital capabilities, potential digital data exchange partners, EHR and other HIE vendor platforms, resident population care needs, technology expertise, and financial and human resource availability. A 'one-size-fits-all' interoperability advancement strategy would be doomed to failure.

[9] Jordan Everson, Wei Chang, Vaishali Patel, Julia Adler-Milstein, The state of health information organizations and plans to participate in the federal exchange framework, *Health Affairs Scholar*, Volume 2, Issue 8, August 2024

PR–8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

a. ***What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?***
b. ***In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually purposed?***
c. ***Are there requirements CMS should consider for data registries to support digital quality measurement in a more efficient manner? Are there requirements CMS should consider for data registries that would support access to real-time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?***

AHCA /NCAL Comment

AHCA/NCAL provided extensive responses in our June 9, 2025 comments in response to the digital quality measure (dQM) concept RFI contained in the FY 2026 SNF proposed payment rule.[10] Below is an excerpt of from our overarching comments regarding common themes scattered throughout our prior RFI responses that we believe are germane to this question: Proceed with Due Diligence; Potential Barriers to Success; Opportunities for and Benefits of Success.

Proceed with Due Diligence

We ask that CMS proceed with due diligence in advancing digital quality measurement in the SNF and not put the cart before the horse. As we discuss elsewhere in this comment letter, due to the historical lack of federal incentives and interoperability implementation support from the HITECH Act and subsequent incentive programs that were directed at hospitals and primary care providers, most SNF and other post-acute and long-term care providers do not have the digital capacity necessary to transition quickly to digital quality measures (dQM) and consideration should be given to foster and reward improvements in digital capacity and not penalize providers that are working towards adding capabilities.

We believe that the SNF QRP program needs to return to its roots as envisioned by the IMPACT Act provisions and focus on those measures that are meaningful, least burdensome, focus on outcomes and not processes, and are scientifically valid to facilitate improvements in care and better assure unbiased comparisons between providers. Such measures should be scalable to both the size of the provider and adequate transition time consideration be given for providers needing to overcome significant gaps in digital capabilities, and reasonable measure exclusions for providers unable to transmit dQM due factors beyond their control. In other words, CMS needs to consider fair and reasonable approaches to overcome the current digital divide status within the SNF sector and to identify approaches to incentivize greater adoption of interoperable technologies including bulk FHIR application programming data export interfaces (APIs).

Potential Barriers to Success

In our detailed responses to the SNF PPS proposed rule RFI questions, we emphasized that many SNF providers do not have the necessary size, human and financial resources, or technical expertise to be able to effectively implement interoperable HIT with any significant level on their own. Many providers remain without an EHR system, let alone the capability to receive, or transmit health records electronically or offer more advanced capabilities such as patient portals or apps. Again, this is the result in part due to factors such as limited or no

[10] AHCA/NCAL Comments: Medicare Program; Prospective Payment System and Consolidated Billing for Skilled Nursing Facilities; Updates to the Quality Reporting Program for Federal Fiscal Year 2026. CMS–1827–P. Submitted, June 9, 2025.

broadband access and lack of financial support or incentives and a lack of coordinated federal governance to support more widespread adoption of interoperable technology.

Additionally, with respect to the concept of converting certain existing SNF assessment-based quality measures to dQM, we believe this merits further exploration as there are several potential benefits that could occur that we discuss below. However, consideration must be given to adjusting the measure specifications to be more dynamic than the currently inflexible measure specifications while assuring the measures remain meaningful and scientifically valid. CMS will need to consider approaches to overcoming these barriers to ensure a successful implementation of dQM processes.

Opportunities for and Benefits of Success

We believe that there are existing SNF assessment-based quality measures that merit exploring as candidates for future transitions to dQM. For example, meaningful and scientifically supported functional outcomes measures. The MDS and the CMS data element library (DEL) also include existing standardized data elements included in FHIR API standards that could be explored for the development of future meaningful dQMs that are not currently feasible under the current inflexible MDS-based assessment measure approach. We discuss in our dQM comments in response to the FY 2026 SNF proposed payment rule how the future measure concept for 'Delirium' might be a possible candidate for a dQM approach as opposed to the current MDS-based measure model.

Beyond the concept of bulk dQM data submission opportunities for success, the more widespread adoption of FHIR APIs across the SNF sector could trigger substantial improvements in the quality of care delivered and clinical outcomes as FHIR flexibilities offer opportunities the share information across disparate HIT systems in a much more efficient and timely process than occurs today, reducing the risk of preventable negative health events and permitting the more timely implementation of interventions. Other benefits of more widespread adoption of FHIR APIs include more comprehensive and timely information being available for providers to enhance their QAPI programs, as well as more timely information that would be available for public reporting purposes.

Providers - Digital Identity

PR–9. How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

- ***a. What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider?***
- ***a. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?***

PR–10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800–63–3 IAL2/AAL2 CSPs):

- ***a. What are the challenges and benefits for providers?***
- ***b. How would requiring their use improve access to health information?***
- ***c. What are the potential downsides?***
- ***d. What impact would mandatory credentials have on a nationwide provider directory?***
- ***e. How could digital identity implementation improve provider data flow?***
- ***f. Would combining FHIR addresses and identity improve data flow?***

PR–11. How could members of trust communities 3 (for example, QHINs, participants and subparticipants in TEFCA, which requires Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs)) better support the goals of reduced provider and patient burden while also enhancing identity management and security?

AHCA /NCAL Comment

AHCA/NCAL provider members agree that a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider would be beneficial in making the patient digital health experience more seamless.

Our provider members understand the importance of protecting resident personal and health information as required under HIPAA provisions. Specific policies and procedures and personnel HIPAA and cybersecurity related training activities are part of the core SNF regulatory compliance activities. However, SNF providers are not technology experts and remain heavily dependent on their IT and digital health product vendors to provide HIPAA-compliant and secure platforms and technologies that also provide capabilities to share information without violating information blocking regulations.

For example, the SNF EHR vendor space is currently dominated by two companies that have some capacity to submit resident assessment data from the Minimum Data Set (MDS) to the CMS iQIES platform on behalf of nursing homes. While there is some progress with these vendors in obtaining certification for specific modules[11], this does not permit seamless interoperability for all relevant health information at this time, even for SNFs that have adopted such certified modules. Currently there are four but soon to be three pathways involving automated submission from the EHR platform or other methods with varying degrees of manual process involvement.

1. SNF personnel manually uploads the MDS to iQIES
2. The SNF EHR directly uploads the MDS to iQIES as programmed.
3. The SNF sends the MDS to a third-party vendor for that vendor to automate the upload to iQIES. In such cases, it is typically through an MDS analytics/scrubber software company.
4. SNF personnel directly enter MDS data into iQIES, which CMS is retiring in October because provider utilization of this method is rare due to redundant data entry burden.

Also, it is the exception rather than the norm for a SNF to routinely exchange the same HIE interoperably with other healthcare system partners and to patients via patient portals or apps. When such exchanges occur, they are usually via customized and often expensive software workarounds. In most cases, the exchange is via photocopy records or fax or in some cases the external partner can log onto the SNF EHR, or vice versa (such as SNF personnel accessing a hospital EHR) to access the information rather than the information being exchanged in an automated manner.

Within Medicare, CMS contractors such as the BFCC QIOs, MACs, and others; health information exchange commonly involves requiring burdensome manual processes. For example, there is a significant burden for claim audits including TPE, SMRC, and CERT. These processes often involve extensive health record documentation. All the associated copying, mailing, uploading, or faxing takes caregivers from the bedside, especially in smaller nursing homes where there is limited to no clerical staff.

A similar limitation is present with state and federal public health reporting, Medicare Advantage payers, and other such external entities that may require non standardized data in different formats including manually entering data directly or loading excel files onto the end-user's portal. Like CMS data systems, many of these

[11] PointClockCare Certifications: https://pointclickcare.com/company/certifications/

portals may have complex data submitter credentialing or multi-factor authentication processes that exacerbate provider burden, when a more seamless interoperability option would be more practical.

Just as healthcare providers needing support for establishing more seamless credentialing of digital identity for interoperable health information exchange, in the absence of coordinated federal support and guidance patients/families/caregivers that have varying levels of digital sophistication in digital security protocols (e.g., logons, user access authorizations, multi-factor authentications) that are secure and HIPAA compliant find todays fragmented digital logon processes too challenging.

Having to establish different digital identical logon credentials to obtain health information from their various healthcare providers and suppliers including physicians, hospitals, SNFs, pharmacies, labs, etc., is intimidating, frustrating and sometimes impossible for public consumers. Establishing a single set of trusted digital identity credentials for digital health products and services that are secure would represent a significant opportunity to make health information availability to a broader range of healthcare providers, provider partners, and patients/families/caregivers in a less burdensome manner. This would help improve the timeliness and appropriateness of care and better quality of care and patient experience of care and satisfaction.

Providers - Information Blocking

PR–12. Should ASTP/ONC consider removing or revising any of the information blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?

AHCA /NCAL Comment

As we have commented in the recent past, AHCA/NCAL does not believe it is appropriate at the time to consider removing or revising any of the information blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) until adequate interoperability support and protections are first established for LTPAC provider settings including our SNF, AL, and ID/DD provider members.[12,13] Technology needs and capabilities vary and 'one-size-fits-all' disincentive policies that would demand that specific health information currently being shared via more manual processes be shared digitally when it may be infeasible in specific provider situations are not helpful. In an era LTPAC providers and their related care delivery partners are actively investing in acquiring secure interoperability capabilities to improve the safety of care and produce better outcomes, federal support and regulations should be directed at facilitating increased digital capabilities, including cybersecurity protections, not imposing bureaucratic barriers such as removing the current reasonable information blocking exceptions that would perversely disincentivize providers from becoming more 'connected'.

PR–13. For any category of healthcare provider (as defined in 42 U.S.C. 300jj(3)), without a current information blocking disincentive established by CMS, what would be the most effective disincentive for that category of provider?

AHCA /NCAL Comment

[12] AHCA/NCAL Comments: Health Data, Technology, and Interoperability: Patient Engagement, Information Sharing, and Public Health Interoperability (HT1-2 RIN 0955-AA06), Submitted October 4, 2024.

[13] AHCA/NCAL Comments: 21st Century Cures Act: Establishment of Disincentives for Health Care Providers That Have Committed Information Blocking Wednesday, November 1, 2023, 88 FR 74947; Centers for Medicare & Medicaid Services (CMS) and Office of the National Coordinator for Health Information Technology (ONC), Department of Health and Human Services (HHS). (RIN 0955–AA05), Submitted January 2, 2024.

As we stated in our response to question PR-12 above, we do not believe it would be appropriate to create inflexible and unattainable added information blocking disincentives for providers seeking to expand their digital health interoperability capabilities. At present, AHCA/NCAL has heard from provider members that patients/family members/caregivers and/or their attorneys approach them accusing them of information blocking despite the provider not having the capacity to share specific health information digitally due to technical limitations. These provider are willing and able to comply with sharing the information through alternative media as is permissible in regulation. The problem often lies in the public awareness of the nuances of the information blocking regulations which we believe that CMS and ASTP/ONC should consider enhancing through expanded education efforts.

We believe there is inherent value for consumers in being able to know what types of digital health information they would be able to access from a provider via a patient portal or patient app, and what type of information would need to be obtained via other media. Such capabilities do vary by individual provider and may change over time. As such, instead of implementing punitive disincentive policies, ASPT/ONC should consider new guidance and education for providers and patients so consumers can make an informed decision about whether they want to receive care from a provider that has more limited digital access to their health information. For example, if a provider has a patient portal or patient app, it would be helpful if they were to inform the patient/family/caregiver about the current capabilities and limits of information available via digital technology. Such an approach would leverage market pressure upon providers to increase their digital health exchange capabilities with patient-facing technologies, instead of taking away those financial resources necessary to invest in technology improvements.

We believe that in establishing standards to implement FHIR APIs, the software application or HIE should be able to and be required to flag which entity that a provider is exchanging data with. For those providers subject to CMS requirements, the technology should seamlessly report this transaction occurrence data to CMS without undue provider intervention. CMS would then be able to populate any relevant public reporting data that would be meaningful to consumers. For example, if a SNF has successfully exchanged data with an admission source hospital within a specified lookback timeframe the hospital would be listed as an interoperable exchange partner for the SNF. This would be the least burdensome approach for SNF providers to demonstrate meaningful interoperability capacity and use and would be more trusted information than obtained and reported via burdensome self-reported SNF attestation statements.

Another example is listing if the SNF has a patient portal or patient app where a resident or their family/responsible party could access or add information to their health information data. This would add some burden as it may require establishing a process for identifying which apps meet all the regulatory safety and security regulations before they can be listed, and for updating what capabilities a specific provider offers for patients/families/caregivers can access digitally through a patient portal or patient app.

PR–14. How can CMS encourage providers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

AHCA /NCAL Comment

AHCA/NCAL believes that such an activity would represent an unfunded burden and would be unproductive. Most information blocking complaints are related to misunderstandings of what information blocking is. As we discussed in our comments in response to question PR-13 above, CMS and ASTP/ONC should be focusing on provider and consumer education about the nuances within the information blocking regulations.

Patients/families/caregivers that are better informed in advance about a potential providers capabilities and limits to share health information digitally and their rights so submit complaints to ASTP/ONC are less likely to

lodge complaints. Any requirement imposed upon a provider to report any situation where a patient/caregiver/family member is dissatisfied would be burdensome and would disincentivize providers from offering patient-facing digital health technology. Patients/families/caregivers need to be made aware that it is not considered information blocking is a provider properly informs them in advance that specific types of information would not be available digitally through their patient portal or app, or occasionally in situations beyond the provider's control such as power outages or internet service provider interruptions.

Technology Vendors, Data Providers, and Networks

Digital Identity

TD–3. Regarding digital identity implementation:

- ***a. What are the challenges and benefits?***
- ***b. How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800–63–3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?***
- c. ***What impact would mandatory use of the OpenID Connect identity protocol have?***

Based on recent developments, strengthening security with additional safeguards in high-profile and high-risk entities where cyberattacks are predominantly occurring should be the number one focus and priority of the health technology ecosystem at all levels (government and private sector). Verifiable digital credentials (VDCs) can help reduce the risk of identity-based cyberattacks; however, "though the concept seems simple, deploying VDCs and understanding their impact on security, privacy and usability in practice can be challenging."[14]

It needs to be kept in mind that the risk profile of smaller health care groups, including SNF, AL, and ID/DD providers, are different than those of larger integrated health systems and centralized integrated health information exchange choke points where the greatest cybersecurity risks related to more streamlined digital identity credentialing might be exploited for cyberattacks. CMS and ASTP/ONC should consider scaling any protections established to the risk profile of the user entering the digital health ecosystem. While some risk is created by insufficient cybersecurity training for providers and patients, most risk of inappropriate access to health information is due to information technology lapses.

Technical Standards and Certification

TD–5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

FHIR endpoints could improve access to health information for patients, providers, and payers by providing a streamlined, centralized data hub for health care directory information that is accurate and verified data. Patients/families/caregivers could quickly retrieve their health records from various providers and payers, allowing them to become more active in their care; easily share health information with new and multiple providers across different health systems, which would improve the continuity of care; and use this information to better understand their conditions, treatment options, and compliance with their care plans.

Providers would also have faster and more efficient access to patient information through a centralized directory, which would lead to improved clinical decision-making and reduced administrative burdens. However, pharmacists and other ancillary health care partners affiliated with SNF, AL, and ID/DD providers are still not

[14] Bill Fisher and Ryan Galluzo, "Digital Identities: Getting to Know the Verifiable Credential Ecosystem," National Institute of Standards and Technology, November 13, 2024. https://www.nist.gov/blogs/cybersecurity-insights/digital-identities-getting-know-verifiable-digital-credential-ecosystem

recognized as providers under Medicare, which is a barrier that must be mitigated to ensure longitudinal patient care provision and patient level data exchange, across the entire digital health ecosystem.

For payers, this would help streamline claim processing and prior authorization, reduce reporting burdens with regulatory agencies and others, and improve network management, ensuring their provider networks are up-to-date.

HL7's National Directory of Healthcare Providers & Services (NDH) Implementation Guide (IG) provides standards and guidance for a national directory infrastructure in the US. Local directories could also use FHIR APIs to make content available to their users.[15] Some federal agencies are further along implementing FHIR capabilities than others.

TD–7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

a. Does it contain the full extent of data elements you need?
b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized?
c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?
d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

The United States Core Data for Interoperability USCDI provides a standardized set of health data classes and data elements for nationwide, interoperable, electronic health information exchange needed by many healthcare providers. As discussed before, due to the historical exclusion of LTPAC providers and their care delivery partner providers and suppliers from incentives implemented through the HITECH Act and subsequent incentive programs available to hospitals and primary care providers, very few SNF, AL, and ID/DD providers have adopted certified EHR module capabilities. ASTP/ONC's USCDI integrates with FHIR US Core, though not all USCDI elements are mapped, and USCDI Data Classes and Element names may differ from FHIR US Core resource names and element names.[16] As stated above in our comments in response to questions PR-5 and PR-6, we believe that the opportunities to exchange health information interoperably from disparate technologies through HIEs and other mechanisms that leverage the more flexible TEFCA governance and FHIR APIs would likely be the preferred approach for our sector to improver interoperability capabilities within the larger digital health ecosystem.

[15] National Directory of Healthcare Providers & Services (NDH) Implementation Guide, HL7. https://build.fhir.org/ig/HL7/fhir-us-ndh/
[16] 3.1 USCDI, HL7 International. https://build.fhir.org/ig/HL7/US-Core/uscdi.html